blood sugar levels spike or are low. The adrenals eventually become exhausted and need support. In individuals with hypoglycemia, dietary and lifestyle changes need to be made with regard to not skipping breakfast and snacking with low glycemic foods during the day.

- Crave sweets during the day
- Irritable if meals are missed
- Depend on coffee to keep going/get started
- Get light-headed if meals are missed
- Eating relieves fatigue
- Feel shaky, jittery, or have tremors
- Agitated, easily upset, nervous
- Poor memory, forgetful between meals
- Blurred vision

Further Assessment of Category X – Sugar Metabolism

Further assessment is required in the following areas:

- Check for diet:
  - Missing meals
  - Sugar, coffee, or fruit for meals
- Check for functional adrenal imbalance

MAF Blood Chemistry Key Association Category X – Sugar Metabolism

- Check for hypoglycemia:
  - Decreased glucose levels
  - Decreased LDH levels

MAF Nutritional Key Category X – Sugar Metabolism*

*Nutritional Support for Sugar Metabolism:**

1. Proglyco-SP™◊ (K13): 1 capsule 3 times a day, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
2. Butyric-FBR™◊ (K133): 2 capsules once a day.

   Alternative Formula
   - Fibromin™◊ (K25): 1 capsule, 2 times a day, or as needed.

Additional Support Considerations:

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 2 tablespoons once a day.§

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

Nutritional support for sugar metabolism should include:

Proglyco-SP™◊ (K13), which supplies the essential nutrients, minerals, and glandulars to support the adrenals and blood sugar metabolism; Adaptocrine™◊ (K2), (K124), which contains herbs that support sugar metabolism and the body's mental functionality under temporary stress; and Super EFA Complex™◊ (K8), which offers a source of essential fatty acids.*

Category XI – Peripheral Utilization of Sugars Interpretation

(Also check for Insulin Resistance)

Insulin resistance refers to a physiological pattern in which insulin receptor sites no longer respond to insulin. Insulin is a hormone that is secreted by the pancreas when serum glucose levels are elevated after meals. Insulin's job is to stimulate receptor sites, which will then transport glucose into the cells.

- Insulin resistance is caused by diets and physiological changes that cause prolonged and frequent release of insulin. Once insulin is repeatedly exposed to receptor sites, the sites will shut down and become nonresponsive. Insulin resistance will then lead to elevations of serum glucose because it cannot be carried into the cell, which leads to alterations in cholesterol and fat metabolism. Insulin resistance places great demands on the beta cells of the pancreas for insulin output, and may eventually lead to diabetes if this is not addressed properly.
- Fatigue after meals
- Crave sweets during the day
- Eating sweets does not relieve cravings for sugar
- Must have sweets after meals
- Waist girth is equal or larger than hip girth
- Frequent urination
- Increased thirst and appetite
- Difficulty losing weight

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

Further Assessment of Category XI – Peripheral Utilization of Sugars

Further assessment is required in the following areas:

- Check for diet:
  - Excess-starch diet
  - Overeating
- Check for elevated cortisol response
- Check for autoimmune issue:
  - Check for late-onset autoimmune diabetes
    - GAD antibodies, islet cell antibodies

MAF Blood Chemistry Key Association Category XI – Peripheral Utilization of Sugars*

- Check for insulin resistance:
  - Increased glucose levels
  - Increased triglycerides levels
  - Increased total cholesterol and LDL levels
  - Decreased HDL levels
  - Increased uric acid levels

MAF Nutritional Key Category XI – Peripheral Utilization of Sugars

*Nutritional Support for the Peripheral Utilization of Sugars:**

1. Glysen™◊ (K1), (K135): 1-2 capsules with meals, or as needed, and/or GlycemoVite™◊ (K85): 1 scoop, with up to 8 ounces of water, between meals.
2. Glycoberine-MX™◊ (K126): 1 capsule with a meal.
   Alternative Formula
   - Glycoberine™◊ Active (K127): 1 capsule with a meal.
3. EnteroVite™◊ (K100): 1-2 capsules with meals.
   Alternative Formula
   - Butyric-FBR™◊ (K133): 1-2 capsules with meals.

Additional Support Considerations:

1. Glysen-Synergy™◊ (K79): 1 packet, once a day, or as needed.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
   Alternative Formula
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
3. Resvero Active™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) 2 times a day.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.
2. Oxicell™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.
   Alternative Formula
   Super Oxicell™◊ (KR23): Apply in the morning, afternoon, and evening.

To provide comprehensive nutritional support for the peripheral utilization of sugars, Glysen-Synergy™◊ (K79) combines four complementary formulas Glysen™◊ (K1), (K135); Adaptocrine™◊ (K2), (K124); Fibromin™◊ (K25); and Protoglysen™◊ (K28) into one easy-to-use package. OmegaCo3™◊ (K7) is a source of essential fatty acids.*

Furthermore, adaptogens, which are herbs that support metabolism of sugar's adrenal feedback loop coordination, such as Adaptocrine™◊ (K2), (K124), are included.* A source of nonsoluble fibers, such as Fibromin™◊ (K25), is included to support gastrointestinal glucose uptake.*

Category XII – Adrenal Interpretation

(Also check for Adrenal Hypofunction)

The adrenal glands are two small glands that sit on top of the kidneys. They produce several important hormones: cortisol to regulate glucose levels, aldosterone to regulate sodium and potassium levels, DHEA to modulate the immune system, and catecholamines to excite the sympathetic nervous system. The adrenal glands are called the anti-stress glands because they allow the body to deal with stress. Stress can be mental/emotional from unhealthy relationships, chemical from inadequate diets (reactive hypoglycemia and food sensitivities), or physical from overtraining. Individuals who present with adrenal exhaustion must avoid caffeine, alcohol, and sugar, as well as any foods to which they may be sensitive.

- Cannot stay asleep
- Crave salt
- Slow starter in the morning
- Afternoon fatigue
- Dizziness when standing up quickly
- Afternoon headaches
- Headaches with exertion or stress
- Weak nails

Further Assessment of Category XII – Adrenal

Further assessment is required in the following areas:

- Check for hypoglycemia lifestyle
- Check for impaired adrenals from:
  - Chronic infection
  - Environmental insult

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- Autoimmunity
- Food sensitivities
- Intestinal permeability
- High-stress lifestyle
- Overtraining

MAF Blood Chemistry Key Association Category XII – Adrenal

- Check for hypofunction:
  - Sodium and potassium changes
- Check Category X on Metabolic Assessment Form™ results to assess sugar metabolism

MAF Nutritional Key Category XII – Adrenal*

*Nutritional Support for Adrenal Function:**

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

*May Also Need to Support Sugar Metabolism (see Category X):**

1. Proglyco-SP™◊ (K13): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

To support adrenal function and sugar metabolism, consider using Proglyco-SP™◊ (K13).* Adaptocrine™◊ (K2), (K124), which is a multi-use herbal formula designed to support a hypothalamic-pituitary-adrenal (HPA) feedback loop should also be considered.* Super EFA Complex™◊ (K8) can be used as a source of essential fatty acids.*

Category XIII – Adrenal* Interpretation

(Also check for Adrenal Hyperfunction)

Adrenal hyperfunction is the initial adrenal response to stress in which the adrenals will increase their release of cortisol. This response is called the alarm reaction and can last for years. Adrenal hyperfunction has the tendency to induce insulin sensitivity and vice versa.

- Cannot fall asleep
- Perspire easily
- Under a high amount of stress
- Weight gain when under stress
- Wake up tired even after 6 or more hours of sleep
- Excessive perspiration or perspiration with little or no activity

Further Assessment of Category XIII – Adrenal

Further assessment is required in the following areas:

- Check for hyperglycemia lifestyle
- Check for impaired adrenals from:
  - Chronic infection
  - Environmental insult
  - Autoimmunity
  - Food sensitivities
  - Intestinal permeability
  - High-stress lifestyle
  - Overtraining

MAF Blood Chemistry Key Association Category XIII – Adrenal

- Check for hyperfunction:
  - Sodium and potassium changes
- Check Category XI on Metabolic Assessment Form™ results to assess the peripheral utilization of sugars

MAF Nutritional Key Category XIII – Adrenal*

*Nutritional Support for Adrenal Function:**

1. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

*May Also Need to Support the Peripheral Utilization of Sugars (see Category XI):**

1. Glysen™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
2. OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.

   Alternative Formula
   - OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

MAF Nutritional Key
Category XIII – Adrenal* (Cont.)

Additional Formula

- Neurosera™◊ (K136): 1-2 teaspoons (5-10 mL) once a day.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

Nutritional support for adrenal function should include Adaptocrine™◊ (K2), (K124), which is a multi-use adrenal formula used to support the HPA feedback control; and Glysen™◊ (K1), (K135), and OmegaCo3™◊ (K7), (K113), which are used to support the peripheral utilization of sugars.*

Category XIV – Electrolyte and pH Balance Interpretation

This section of the manual is related to electrolyte and pH balance. Imbalances may occur secondary to diet, exercise, hydration, renal imbalances, adrenal imbalances, dysglycemia, and pulmonary imbalances. Careful screening for related mechanisms should be conducted if symptoms are present.

- Edema and swelling in ankles and wrists
- Muscle cramping
- Poor muscle endurance
- Frequent urination
- Frequent thirst
- Crave salt
- Abnormal sweating from minimal activity
- Alteration in bowel regularity
- Inability to hold breath for long periods
- Shallow, rapid breathing

Further Assessment of Category XIV – Electrolyte and pH Balance

Further assessment is required in the following areas:

- Check for tired/sluggish
- Check for cold—hands, feet, all over
- Check for excess acidifying diet
- Check for overhydration
- Check for physiological failure in pH buffering system
- Check for physiological imbalance in metabolism of sodium, potassium, chloride, calcium, or magnesium

MAF Blood Chemistry Key Association
Category XIV – Electrolyte and pH Balance

- Check for sodium, potassium, chloride, calcium, and magnesium levels
- Check for CO_2 levels
- Check for anion gap

MAF Nutritional Key
Category XIV – Electrolyte and pH Balance*

*Nutritional Support for Electrolyte and pH Balance:**

1. Electro-pH™◊ Complex (K86): 1 scoop with up to 8 ounces of water.

Category XV – Thyroid Interpretation

(Also check for Hypofunction)

The thyroid gland is in charge of two main functions. First, it controls the rate of metabolism in the body, and second, it helps regulate calcium levels in the bone through the hormone calcitonin. The symptoms of low thyroid function may originate from numerous alterations in metabolism, such as a decrease in thyroid or pituitary hormone synthesis, altered hypothalamic-pituitary-feedback coordination, altered thyroid hormone transport, altered peripheral hormone metabolism, hormone detoxification, thyroid receptor-site resistance, etc.

- Tired/sluggish
- Feel cold—hands, feet, all over
- Require excessive amounts of sleep to function properly
- Increase in weight even with low-calorie diet
- Gain weight easily
- Difficult, infrequent bowel movements
- Depression/lack of motivation
- Morning headaches that wear off as the day progresses
- Outer third of eyebrow thins
- Thinning of hair on scalp, face, or genitals, or excessive hair loss
- Dryness of skin and/or scalp
- Mental sluggishness

Further Assessment of Category XV – Thyroid

Further assessment is required in the following areas:

- Check for Hashimoto's response
- Check for goitrogenic impairment
- Check for T4-to-T3 conversion impairment

- Check for pituitary TSH release impairment
- Check for thyroid-binding globulin imbalance
- Check for metabolic syndrome

MAF Blood Chemistry Key Association Category XV – Thyroid

- Check for hypofunction:
  - Increased TSH levels
  - T3 uptake
  - Total T4 levels
  - Total T3 levels
  - FTI
  - Increased thyroid antibodies

MAF Nutritional Key Category XV – Thyroid*

*Nutritional Support for Thyroid Function:**

1. Thyroxal™◊ (K12): 1-2 capsules with meals.

*Nutritional Support for Thyroid Immune Pathways:**

1. Thyro-FLM™◊ (K137): 2 capsules once a day.

*Nutritional Support for the Thyroid:**

1. Thyro-CNV™◊ (K9): 1-2 capsules with meals.

*Nutritional Support for T3:**

1. Thyro-CNV™◊ (K9): 1-2 capsules with meals.
2. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.

Also Consider:

1. AdrenaCalm™◊ (KR16), (KR73): Apply in the morning, afternoon, and evening.

Category XVI – Thyroid Interpretation

(Also check for Hyperfunction)

There are different degrees of overactive thyroid function. Some may be managed functionally, and some may need the intervention of medications and hormones for a brief period of time. The main culprit of an overactive thyroid includes some type of antigen that triggers an inflammatory or autoimmune response. The antigen may be a virus, a food sensitivity, a gut pathogen that has altered immune responses, etc.

- Heart palpitations
- Inward trembling
- Increased pulse even at rest
- Nervous and emotional
- Insomnia
- Night sweats
- Difficulty gaining weight

Further Assessment of Category XVI – Thyroid

Further assessment is required in the following areas:

- Check for Hashimoto's response
- Check for excessive thyroid hormone intake
- Check for neurochemical imbalance
- Check for autoimmune issue:
  - GAD antibodies

MAF Blood Chemistry Key Association Category XVI – Thyroid*

- Check for hyperfunction:
  - Decreased TSH levels
  - T3 uptake
  - Total T4 levels
  - Total T3 levels
  - FTI
  - Increased thyroid antibodies

MAF Nutritional Key Category XVI – Thyroid*

*Nutritional Support for the Thyroid:**

1. Trizomal™◊ Glutathione (K122), (K129A): 5 mL (approx. 1 teaspoon or 1 packet) 1-2 times a day.
   Alternative Formulas
   - AC-Glutathione 250™◊ (K117): 1 capsule 1-2 times a day.
   - AC-Glutathione™◊ (K88): 1-2 capsules 1-2 times a day.
2. ClearVite™◊ Formula (K84, 95, 105, 108, 118, or 142): As directed by the ClearVite™◊ Basic Program Guide.
3. Nitric Balance™◊ (K62), (K68), (K82): 1 teaspoon once a day.

Additional Support Considerations:

1. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
2. Ultra-D 5000™◊ (K78): 1-4 teaspoons, once a day, with meals.

For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

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MAF Nutritional Key Category XVI – Thyroid* (Cont.)

Alternative Formulas

- Ultra-D Complex™◊ (K35): 1-4 teaspoons, once a day, with meals.
- AD-Pro™◊ (K119): 1 softgel once a day.
- ADEK-Pro™◊ (K102): 1-3 softgels once a day.

For additional vitamin D, consider Liqua-D™◊ (K87): 1 drop once a day.

§Currently unavailable

Also Consider:

Oxicell™◊ (KR22), (KR70): Apply in the morning, afternoon, and evening.

Alternative Formula

- Super Oxicell™◊ (KR23): Apply in the morning, afternoon, and evening.

There are three products to naturally support the thyroid: Thyroxal™◊ (K12), Thyro-CNV™◊ (K9), and AC-Glutathione™◊ (K88), (K117).* Thyroxal™◊ (K12) is a multi-use nutritional, mineral, herbal, and glandular support for thyroid function that does not contain iodine or tyrosine.* Additional iodine support for the thyroid is contraindicated in individuals who consume a high-salt diet. The amino acid tyrosine is contraindicated in individuals suffering from a stress response. Thyro-CNV™◊ (K9) is a multi-use vitamin, mineral, and herbal product designed to support the conversion of peripheral T4 to T3.* AC-Glutathione™◊ (K88), (K117) offers 125 mg per capsule of S-acetyl L-glutathione, which is an alternative form of reduced glutathione.

Category XVII – Prostate Interpretation (Males Only)

The prostate is a small, chestnut-shaped gland that lies below the bladder and surrounds the urethra. Nonmalignant enlargement of the prostate gland is called benign prostatic hypertrophy (BPH) and is characterized by symptoms of bladder obstruction, such as increased urinary frequency, nighttime awakenings to empty the bladder, and reduced force and caliber (speed of flow) of urination. The key issue with BPH is to differentiate BPH from the more serious prostate cancer. If the clinician is suspicious of malignancy, follow-up diagnostic evaluations, such as the manual prostate exam, a prostate-specific antigen (PSA) and free PSA test, and even an ultrasound to make the differentiation by a qualified urologist should be performed.

- Urination difficulty or dribbling
- Frequent urination
- Pain inside of legs or heels
- Feeling of incomplete bowel emptying
- Leg twitching at night

Further Assessment of Category XVII – Prostate

Further assessment is required in the following areas:

- Clinician needs to consider the possibilities of benign prostatic hyperplasia or prostate cancer so check for the following:
  - Prostate-specific antigen (PSA)
  - Free PSA
  - Imaging

MAF Blood Chemistry Key Association Category XVII – Prostate

- Check for PSA levels
- Check for free PSA levels

MAF Nutritional Key Category XVII – Prostate*

*Nutritional Support for the Prostate:**

1. Prosta-DHT™◊ (K6): 1-2 capsules with meals.

Additional Support Considerations:

1. Fibromin™◊ (K25): 1-3 capsules with meals.
2. Estrovite™◊ (K5): 1-2 capsules with meals.
3. Bladder Serene™◊ (K138): 1 scoop (2.6 g), with up to 6 ounces of water, once a day.
4. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas

   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

To provide nutritional support for the prostate, consider using Prosta-DHT™◊ (K6), Super EFA Complex™◊ (K8), and Estrovite™◊ (K5).* Prosta-DHT™◊ (K6) is a multi-use nutritional, herbal, and amino acid complex to support testosterone levels.* Super EFA Complex™◊ (K8) offers a wide variety of essential fatty acids required to support the prostate.* DHT, which is responsible for prostate tissue hyperplasia, is unable to be excreted from the prostate and therefore contributes to hyperplasia with elevated estrogen levels. Estrovite™◊ (K5) is a multi-use nutritional and herbal formula to support estrogen metabolism and detoxification.*

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A source of supplemental fiber to support the amount of sex hormone-binding globulin can be found in Fibromin™◊ (K25).*

Category XVIII – Andropause (Males Only) Interpretation

This category is a quick screen for the typical symptoms of andropause. These symptoms take place when there are low androgen levels, such as testosterone, DHT, or androstenedione. These symptoms can also take place if there is a low testosterone-to-estrogen ratio. This ratio manifests when there is a state of estrogen dominance. If the individual has numerous symptoms in this category, andropause should be considered. There are many factors and systems involved in the patterns associated with andropause. These factors include: liver detoxification, estrogen metabolism, gastrointestinal microflora activity, insulin sensitivity, adrenal function, testosterone synthesis, 5-alpha reductase activity, beta-glucuronidase activity, 17,20-lyase activity, aromatase activity, and the feedback loops involved with the pituitary and the hypothalamus. A male hormone profile that includes measurements of testosterone, androstenedione, DHEA, progesterone, estrone, LH, and FSH can be a very useful and accurate tool to identify the patterns of andropause in addition to the andropause questionnaire.

- Decreased libido
- Decreased number of spontaneous morning erections
- Decreased fullness of erections
- Difficulty maintaining morning erections
- Spells of mental fatigue
- Inability to concentrate
- Episodes of depression
- Muscle soreness
- Decreased physical stamina
- Unexplained weight gain
- Increase in fat distribution around chest and hips
- Sweating attacks
- More emotional than in the past

Further Assessment of Category XVIII – Andropause (Males Only)

Further assessment is required in the following areas:

- Check for insulin surge promotion of aromatase
- Check for inflammatory compromise of Leydig cells
- Check for stress-response suppression of LH
- Check for Dihydrotestosterone (DHT) elevation, leading to DHT receptor-site resistance

MAF Blood Chemistry Key Association Category XVIII – Andropause (Males Only)

- Check for insulin resistance
- Check for systemic inflammation
- Check for GI infections
- Check for neurodegeneration
- Check hormone levels (testosterone:estrogen ratio)

MAF Nutritional Key Category XVIII – Andropause* (Males Only)

*Nutritional Support for Testosterone Levels:**

1. Opticrine™◊ (K3): 1-2 capsules with meals.
2. DHEA-25C™◊ (K141): 1 capsule once a day.
   Alternative Formula
   - DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

*Nutritional Support for Estrogen Levels:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.
2. Glysen™◊ (K1), (K135): 2-3 capsules, with meals, or as needed.
3. OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   Alternative Formula
   - OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.
4. Fibromin™◊ (K25): 1 capsule, 2 times a day, or as needed.
5. Resvero Active™◊ (K76), (K110): 5 mL (approx. 1 teaspoon) once a day.
   Alternative Formula
   - Resvero-SE™◊ Active (K125): 1 softgel once a day.

Also Consider:

1. Testanex™◊ (KR17), (KR74): Apply 2 times a day.

Many nutritional supplements can be effective in supporting a male hormone balance.* For example, Opticrine™◊ (K3) can be used to support testosterone synthesis.* The peripheral utilization of sugars can contribute to excess estrogen levels by upregulating aromatase. Glysen™◊ (K1) and OmegaCo3™◊ (K7), (K113) are essential supports for the peripheral utilization of sugars.* A source of supplemental fiber to support sex hormone-binding globulin levels can be found in Fibromin™◊ (K25).*

Category XIX – Menstruating Women Interpretation

The female reproductive cycle is very sensitive, even to slight alterations in hormone metabolism. Alterations in

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.

menstruation should give the clinician clues as to which systems may need support.

- Perimenopausal
- Alternating menstrual cycle lengths
- Extended menstrual cycle (greater than 32 days)
- Shortened menstrual cycle (less than 24 days)
- Pain and cramping during periods
- Scanty blood flow
- Heavy blood flow
- Breast pain and swelling during menses
- Pelvic pain during menses
- Irritable and depressed during menses
- Acne
- Facial hair growth
- Hair loss/thinning

Further Assessment of Category XIX – Menstruating Women

Further assessment is required in the following areas:

- Check for blood glucose issues
- Check for liver detoxification need
- Check for essential fatty acid imbalances

MAF Blood Chemistry Key Association Category XIX – Menstruating Women

- Check for systemic inflammation
- Check hormone levels

MAF Nutritional Key Category XIX – Menstruating Women*

*Nutritional Support for Progesterone Function:**

1. Progestaid™◊ (K4): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.

*Nutritional Support for Adrenal Function:**

1. Proglyco-SP™◊ (K13): 1-2 capsules with meals.
2. Adaptocrine™◊ (K2), (K124): 1-3 capsules with meals.
3. OmegaCo3-SE™◊ (K113): 1-2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1-2 tablespoons 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

Also Consider:

1. AdrenaStim™◊ (KR15), (KR72): Apply in the morning and at noon.

§Currently unavailable

Progestaid™◊ (K4) is a natural herbal and nutritional product to support luteal function and progesterone output.*
Estrovite™◊ (K5) is a multi-use vitamin, mineral, and herbal product to support estrogen receptor activity, detoxification of estrogens, and estrogen metabolism.*
Super EFA Complex™◊ (K8) offers the female a variety of essential fatty acids to support female hormone metabolism.* To support adrenal function and hormone production, women can use Adaptocrine™◊ (K2), (K124), and Proglyco-SP™◊ (K13).*

Category XX – Menopausal Women Interpretation

The symptoms of menopause can be related to numerous functional imbalances. A comprehensive female hormone profile may be required for specific nutritional protocols. However, some nutritional products may prove to be helpful in most cases.

- How many years menopausal
- Since menopause any uterine bleeding
- Hot flashes
- Mental fogginess
- Disinterest in sex
- Mood swings
- Depression
- Painful intercourse
- Shrinking breasts
- Facial hair growth
- Acne
- Increased vaginal pain, dryness, or itching

Further Assessment of Category XX – Menopausal Women

Further assessment is required in the following areas:

- Check for blood glucose issues
- Check for liver detoxification need
- Check for essential fatty acid imbalances

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Apex Energetics™ products are intended for nutritional use and health maintenance only and should not replace or delay the use of any medication.

MAF Blood Chemistry Key Association Category XX – Menopausal Women

- Check for systemic inflammation
- Check hormone levels

MAF Nutritional Key Category XX – Menopausal Women*

*Nutritional Support for Progesterone Function:**

1. Progestaid™◊ (K4): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§

§Currently unavailable

*Nutritional Support for Estrogen Metabolism:**

1. Estrovite™◊ (K5): 1-2 capsules with meals.
2. OmegaCo3-SE™◊ (K113): 2 softgels 1-2 times a day.

   Alternative Formulas
   - OmegaCo3™◊ (K7): 1 tablespoon 1-2 times a day.
   - Super EFA Complex™◊ (K8): 1-2 tablespoons with meals.§
3. DHEA-25C™◊ (K141): 1 capsule once a day.

   Alternative Formula
   - DHEA (K18): 10 drops (0.255 mL) 1-2 times a day.

§Currently unavailable

*Nutritional Support for Progesterone Function:**

1. Pregnenolone (K19): 10 drops (0.255 mL) 1-3 times a day.

Progestaid™◊ (K4) is a natural herbal and nutritional product to support luteal function and progesterone output.* Estrovite™◊ (K5) is a multi-use vitamin, mineral, and herbal product to support estrogen receptor activity, support a detoxification of estrogens, and support estrogen metabolism.* Super EFA Complex™◊ (K8) offers a variety of essential fatty acids to support female hormone metabolism.*

Part III: Foods

This category screens for foods that have both a positive and negative impact on metabolism, such as essential fatty acids, lignans, dietary fiber, caffeine, and partially hydrogenated fats. The individual's consumption of raw nuts, seeds, and fish will give the clinician a general idea of the amount of essential fatty acids (EFAs) in his/her diet. The clinician needs to clarify that toasted or flavored nuts, canned tuna, and fried fish do not count as a source of EFAs. If the individual has a low dietary intake of EFAs, an EFA supplement should be recommended. Most individuals will require an omega-3 supplement versus an omega-6 source.

The individual's consumption of vegetables needs to be identified. Vegetables are necessary to provide a source of fiber and lignans that are crucial in managing metabolism. These substances have a positive impact on the gastrointestinal tract and its influence on hormone metabolism. This is especially true in elevated hormone patterns. The source of dietary fibers from plants has the potential to inhibit the activity of gastrointestinal beta-glucuronidase, which deconjugates estrogens conjugated in the liver.

The enzyme beta-glucuronidase is found in the large intestine and has the potential to allow hormones to be reabsorbed and remetabolized if the diet is low in fiber. Cruciferous vegetables also contain a chemical component called indole-3-carbinol, which can promote C-2 hydroxylation over C-4 and/or C-16 hydroxylation of estrogens. This will shift estrogen metabolism ratios to 2-hydroxyestrone, which has low estrogenic activity compared to 4-hydroxyestrone or 16 alpha-hydroxyestrone, which has powerful estrogenic activity. Fiber and lignans in the diet also increase the circulating concentrations of sex hormone-binding globulin (SHBG), thus reducing levels of unbound, active hormones.

The consumption of caffeine can have an adverse impact on metabolism at multiple levels. First, caffeine is an adrenal stimulant and can promote overactivity and shift pregnenolone away from androgen synthesis into glucocorticoid synthesis. Second, caffeine increases the demand placed on hepatic detoxification and can deplete the substrates required for hormone detoxification by the cytochrome P450 family. Third, caffeine can have an adverse impact on gastrointestinal health. If an individual suffers from metabolic alterations and is not willing to stop his/her intake of caffeine, he/she will respond very slowly. Please note that decaffeinated coffee still contains 60% caffeine, and is not an acceptable alternative. It is important to educate individuals that their dependence on caffeine may be resolved if they support their adrenal glands and stabilize their glucose levels during the day.

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For treatment of this condition: Seek appropriate conventional medical management and drug administration by a physician.